

WEIGHT LOSS

Titration & Dose Escalation Management

One set of advance, hold and reduce criteria governing every agent the practice prescribes

DOCUMENT WL-004	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Agent-specific ladders live in WL-001 and WL-002. This protocol governs the decision made at every step of those ladders: advance, hold, or reduce. Applying one written rule across all prescribers is what makes the program consistent and the chart defensible.

ESCALATION IS A CLINICAL DECISION

It is not a scheduled event, a subscription tier, or a response to disappointment. If the reason recorded for an escalation is that four weeks elapsed, the decision has not actually been made.

2 Decision Criteria

ASSESS ALL FOUR BEFORE DECIDING

Domain	Question
Interval	Has the minimum duration at the current dose been completed?
Tolerance	Are GI and other adverse effects absent or mild and stable (WL-005)?
Response	Is progress inadequate relative to the expected trajectory for this dose and duration?
Adherence and intake	Are doses being taken as prescribed, and is oral intake, protein and hydration adequate (WL-007)?

Advance

- Minimum interval completed at the current dose.
- Adverse effects absent, or mild and not worsening.
- Response inadequate against the agreed goal and the expected trajectory.
- Adherence confirmed and nutritional intake adequate.
- Current dose is below the labeled maximum.
- No new contraindication, medication or condition since the last review.

Hold at current dose

- Response is adequate and the patient is progressing toward the goal.

- Adverse effects are present at moderate grade or are not yet stable.
- Interval not completed.
- Adherence uncertain, or intake and protein inadequate — fix these before adding dose.
- Recent intercurrent illness, dehydration, or a planned procedure requiring sedation.
- Patient preference to hold, once the implications have been discussed and recorded.

Reduce or pause

- Moderate adverse effects that are persistent, worsening, or affecting daily function.
- Any severe adverse effect (WL-005), pending evaluation.
- Weight loss occurring alongside poor oral intake, dehydration, or apparent lean mass loss.
- Signs of an emerging disordered eating pattern — pause and refer per WL-003.
- Intercurrent illness with reduced intake, or hospital admission.
- Patient unable to attend monitoring — therapy is paused rather than continued unobserved.

3 Managing Requests to Escalate Faster

This is the most common friction point in a weight-loss program and the most common source of protocol drift. Handle it the same way every time.

- 1 **Acknowledge the goal** without agreeing to the method. The patient's frustration is usually real and reasonable.
- 2 **Explain the reason for the interval** — tolerance, adverse effect risk, and the fact that faster escalation does not reliably produce faster loss.
- 3 **Review the actual data together:** trajectory to date, adherence, intake and protein. The limiting factor is frequently not the dose.
- 4 **Address the modifiable factors** per WL-007 and set a defined reassessment point.
- 5 **Decline clearly** where criteria are not met. An unclear answer will be read as a maybe and re-asked next week.
- 6 **Document the request, the discussion and the decision.** A declined escalation is one of the strongest entries a weight-loss chart can contain.

NEVER ESCALATE OUTSIDE THE LABELED MAXIMUM

Requests to exceed the maximum labeled dose, to stack agents, or to source additional product independently are declined without exception. Where a patient indicates they intend to obtain product elsewhere, document the counseling given and the risks explained, and notify the medical director.

4 Documentation

Each escalation decision — including a decision not to escalate — is recorded with the four domains assessed. This takes two lines and it is the difference between a titration that was reasoned and one that merely happened.

- Current dose, weeks at that dose, and the date of the last change.

- Weight and percentage change since the last review and since baseline.
- Adverse effect grading at the time of the decision.
- Adherence and intake assessment.
- Decision taken and the criteria supporting it.
- Any patient request that differed from the decision, and the discussion held.
- Next review date.

5 Quick Reference

QUICK REFERENCE CARD

Advance · Hold · Reduce

1. **Check all four:** interval, tolerance, response, adherence and intake.
2. **Advance** only if interval done, tolerance good, response inadequate, below max.
3. **Hold** if responding well, symptoms unsettled, or intake inadequate.
4. **Reduce** for persistent moderate or any severe adverse effect.
5. **Pause** for illness, poor intake, missed monitoring, or eating-disorder signals.
6. **Never exceed the labeled maximum.** Never stack agents.
7. **Record the decision either way** — a documented 'no' is worth more than a 'yes'.

Print, laminate and post at the point of care

6 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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